ALICE L WALTON SCHOOL OF MEDICINE

Mitral Stenosis

Last updated February 20, 2026 17 min read
CC BY-SA 4.0

Adapted by: Diptiman Bose, Patrick Brooks, Phil Riley, Ian Murray
Editors: Chelsey Deel, Jordan Finn, Russell Johnson

Adapted from: Mitral Stenosis

Listen to this Brick
0:00 / 26:02 1x

Learning Objectives (4) Versions

After completing this brick, you will be able to:

1. Define mitral stenosis and its clinical presentation.
2. Explain the etiology, pathophysiology, and complications of mitral stenosis.
3. Explain the diagnosis of mitral stenosis.
4. Describe the

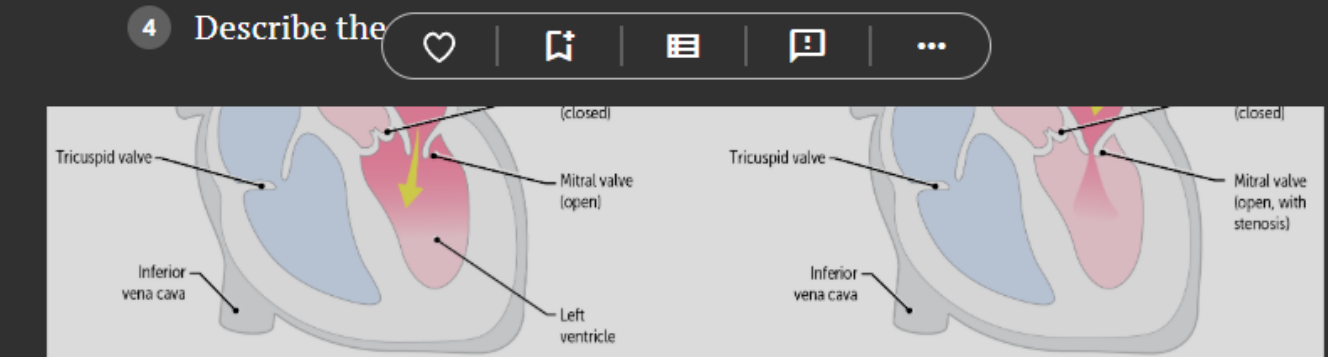
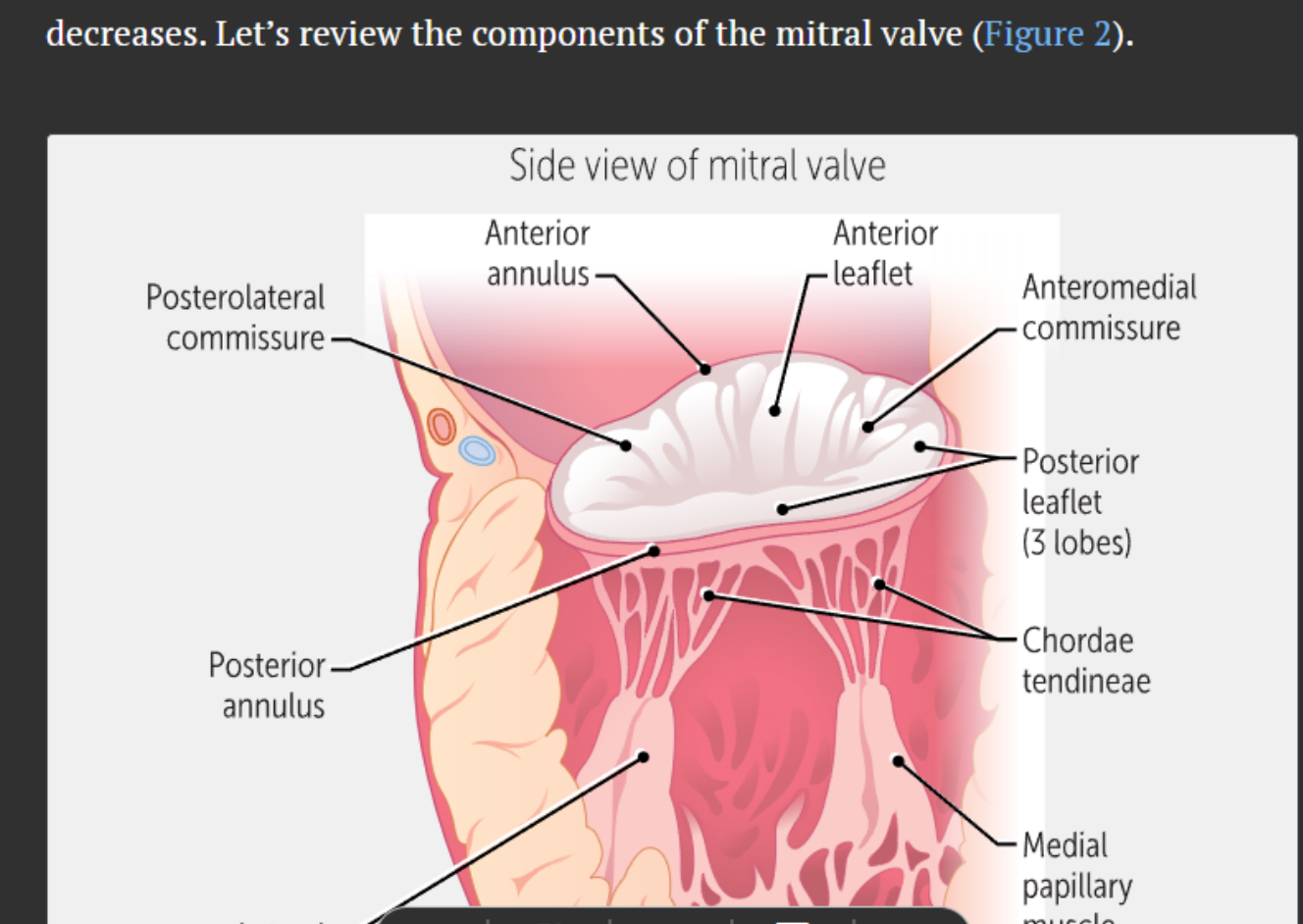


Figure 1 Mitral stenosis anatomy

Excess volume and pressure build up in the LA, while forward flow to the LV decreases. Let's review the components of the mitral valve (Figure 2).



may not always be aware of their symptoms, so taking a good history is important. Patients with either MS or Mitral Regurgitation may present with new onset atrial fibrillation caused by the atrial dilation (discussed below). The normal hemodynamic changes seen in pregnancy (increased heart rate and blood volume) may worsen symptoms or unmask previously asymptomatic MS (also discussed below).

Patients with MS may also experience hemoptysis. Increased pressure in the pulmonary circulation can cause capillaries in the lungs to rupture. Hemoptysis due to MS is less common now, due to earlier detection and management.

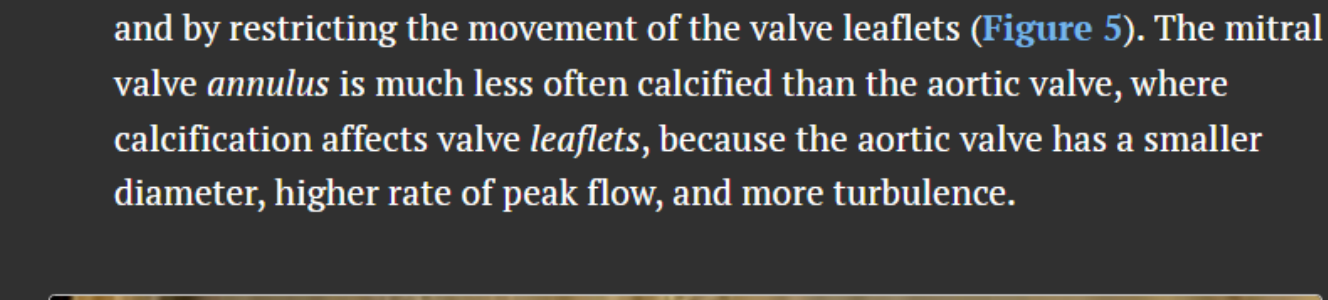
> CLINICAL CORRELATION
>
> Mitral Stenosis is more likely to cause hemoptysis than Mitral Regurgitation. Although both cause elevations in pulmonary vascular pressures which can lead to damage and ultimately bleeding from fragile pulmonary blood vessels, MS causes higher and more consistent elevations due to reduced flow across the mitral valve resulting in persistently greater left atrial pressures than MR.

What Is the Pathophysiology of Mitral Stenosis? (LO 2)

MS is caused by a variety of diseases that narrow the mitral valve. For comparison, a healthy cadaveric mitral valve is shown (Figure 3). Note the healthy **commissures** (ALC and PMC), the points where adjacent leaflets meet and join together. Th lve leaflet function, allowing them to separate and open/close properly. When these areas become

valve is the most common heart valve involved in RHD. Rheumatic fever is an inflammatory condition that can develop after inadequate treatment of streptococcal infections caused by group A streptococcus (GAS). It triggers an immune response to the antigens on GAS which are very similar to the structure of valvular tissue, resulting in autoimmune attack of the heart valves. Although the prevalence of rheumatic heart disease in developed nations like the United States is on the decline, an estimated 1 in 100,000 patients still contract this disease. Initially, rheumatic fever may present with Mitral Regurgitation. However, over a period of years to decades, chronic inflammation of the mitral valve leads to thickened, calcified leaflets of the valve with fibrin deposits on the cusps causing stenosis. Severe inflammation results in **commissural fusion** - the leaflets fuse together at the commissures. As a result, the stenotic mitral valve from rheumatic disease develops a characteristic slit-like, curved opening that resembles a fish mouth (Figure 4).

- **Calcification of Annulus.** In older patients calcium can deposit along the annulus. Annular calcification more commonly interferes with leaflet closure and results in Mitral Regurgitation. In advanced cases it can cause Mitral Stenosis by limiting the annulus's ability to expand during diastole and by restricting the movement of the valve leaflets (Figure 5). The mitral valve *annulus* is much less often calcified than the aortic valve, where calcification affects valve *leaflets*, because the aortic valve has a smaller diameter, higher rate of peak flow, and more turbulence.



Complications of Mitral Stenosis (LO 2)

Most complications of MS are related to structural changes to the LA that lead to atrial fibrillation (AF) and thromboembolism.

Atrial Fibrillation (A fib). MS (and MR) may cause A fib, an irregularly irregular heart rhythm, in 30%-70% of patients. This is due to enlargement of the LA, which dilates to accommodate the extra blood volume. As the LA expands, the normal architecture of the heart wall is stretched, disrupting its electrical activity and increasing the risk of A fib.

Thromboembolism. In atrial dilation and A fib, the lack of regular atrial contractions allows blood to remain static in the LA. This increases the risk of formation of blood clots (atrial thrombi). This can lead to thromboembolism, in which a blood clot travels downstream from the LA and exits the heart to occlude the arterial blood supply. This occlusion occurs most often in a part of the cerebral vasculature, resulting in a **cardioembolic stroke** (Figure 7).

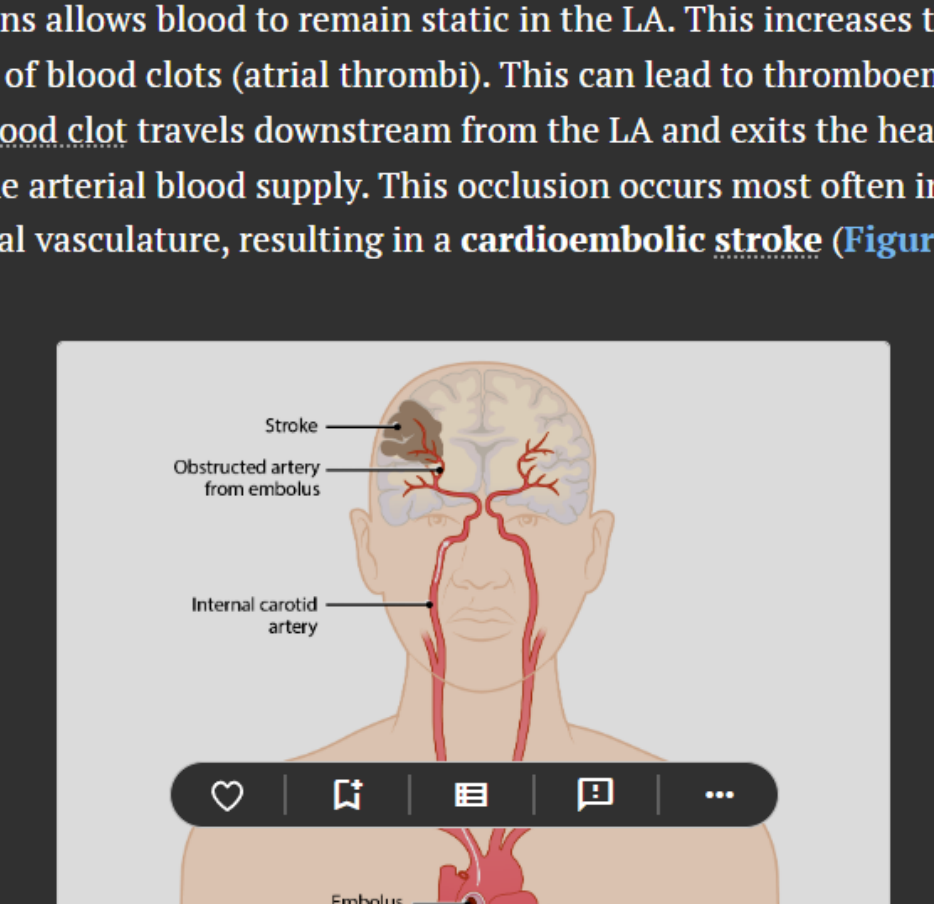

Stroke
Obstructed artery from embolus
Internal carotid artery
Embolus
Atrial fibrillation (blood clot formation)
Heart

Figure 7. Atrial Fibrillation resulting in Cardioembolic Stroke

Heart Failure. Although MS does not directly injure the LV, severe MS will lower preload, reducing cardiac output. So patients may develop some symptoms of reduced cardiac output seen in heart failure, such as fatigue or dizziness.

Right heart failure can sometimes be seen in severe MS, as the increased pulmonary capillary pressures then increase pulmonary artery and RV pressures. This may, in severe cases, lead to findings of right heart failure like jugular venous distension, lower extremity edema, and hepatomegaly.

Complications in Pregnancy. Normal hemodynamic changes of pregnancy include a 30 - 50% increase in blood volume and a 20% increase in heart rate. The increased blood volume is deleterious in MS because a larger volume of blood is trying to get through the stenotic mitral valve, and this can lead to pulmonary edema. The increase in heart rate is also poorly tolerated, because diastole shortens with increasing heart rate, further impairing ventricular filling. Previously asymptomatic patients will often present with symptoms of MS during their pregnancy. The dyspnea and tachycardia can be dangerous to pregnant patients.

> CLINICAL CORRELATION
>
> Rheumatic **Mitral Stenosis** is the most common cardiac condition found in women during pregnancy. It is poorly tolerated due to the large increase in intravascular volume and increase in heart rate. Beta blockade can slow the heart rate and improve symptoms.
>
> Severe **Aortic Stenosis** is also poorly tolerated during pregnancy, since the cardiac output must increase significantly to perfuse the placenta and support the growing fetus. Increasing cardiac output in the setting of a narrowed Aortic Valve puts strain on the left ventricle.
>
> **Mitral Regurgitation** is better tolerated during pregnancy. The large increase in preload is offset by the decreased peripheral vascular resistance of pregnancy (estrogen and progesterone promote vasodilation, and the placental circulation is low-resistance). By lowering vascular resistance, the LV can eject more blood into the aorta and less blood back into the left atrium, decreasing the severity of the regurgitation.

Why do pregnant patients with MS tend to present symptomatically?

How Do We Diagnose Mitral Stenosis? (LO 3)

MS is typically suspected because of findings during physical examination, but it is definitively diagnosed with an echocardiogram.

Physical Examination

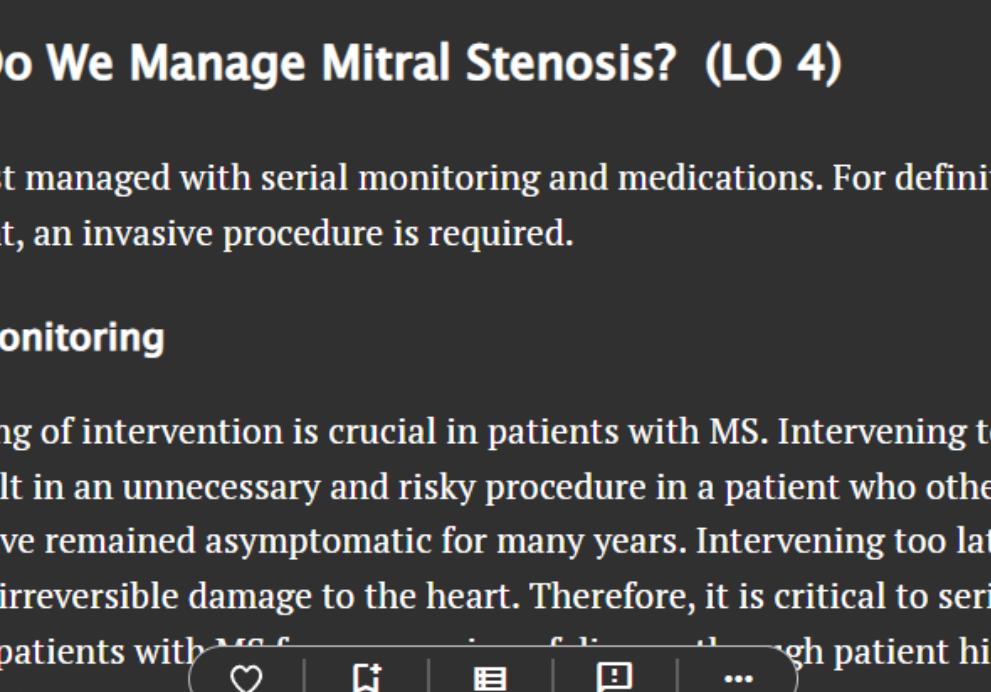
In patients with MS, cardiac auscultation may reveal a **diastolic murmur** as blood flows through the valve. This is often described as a low-pitched diastolic rumble. The murmur is best heard at the apex with the patient lying in a left lateral recumbent position.

An audible **opening snap** of the mitral valve is characteristic of MS, which is created by an abrupt, forceful opening of the valve under high left atrial pressure. The stiff leaflets abruptly halt at their maximal opening position with the sudden tensing resulting in the sound. It is heard immediately after S2 (second heart sound), early in diastole. The opening snap is followed by the **diastolic decrescendo-crescendo** murmur. It slows during passive ventricular filling then increases with atrial contraction-this is also referred to as **pre-systolic accentuation.** Presystolic accentuation disappears when atrial fibrillation is present, as the atrial kick is lost.

Why is an opening snap heard in mitral stenosis?

Imaging

Transthoracic echocardiogram (TTE) is indicated to establish a diagnosis by visualizing the stenotic valve orifice of Mitral Stenosis and to determine severity. A transesophageal echocardiogram (TEE) allows for better visualization of the LA and mitral valve as the ultrasound probe is inserted through the esophagus, which is nearer the LA. A TEE is a more invasive procedure, requiring sedation.



For the above video, use the gear to slow down the playback settings.

Electrocardiography

An ECG is not critical to diagnosing MS but may show LA enlargement and/or Atrial Fibrillation.

How Do We Manage Mitral Stenosis? (LO 4)

MS is best managed with serial monitoring and medications. For definitive treatment, an invasive procedure is required.

Serial Monitoring

The timing of intervention is crucial in patients with MS. Intervening too early may result in an unnecessary and risky procedure in a patient who otherwise would have remained asymptomatic for many years. Intervening too late may result in irreversible damage to the heart. Therefore, it is critical to serially monitor patients with gh patient history and periodic TTE.

Medical Therapy

The first goal of medical treatment of MS is to control heart rate, typically with **β-blockers.** Why is this important? MS is an obstruction during diastole that inhibits passive filling of blood from the LA to the LV. As heart rate increases and the cardiac cycle shortens, nearly all of the time is subtracted from the diastolic phase. Slowing the heart rate allows more time for blood to flow through during diastole. Therefore, β-blockers work to reduce symptoms; however, they do not treat the underlying condition.

Because of the high risk of thromboembolism (>6% per year), patients with A fib and MS should take **warfarin** (an anticoagulant). The same applies to patients with MS and LA thrombus or a prior history of an embolism (eg, stroke).

Patients with volume overload (pulmonary edema) should be treated with **diuretics.**

Invasive Procedures

An invasive procedure is the only definitive treatment for MS. During **percutaneous mitral balloon commissurotomy** (aka valvotomy), a balloon tipped catheter is inserted via the femoral vein into the right atrium. A transeptal puncture then advanced the catheter to the left atrium and then across the mitral valve. The valve is dilated by a balloon (Figure 9). Percutaneous intervention is done in those with severe MS who have favorable valvular anatomy as determined by an echocardiographic scoring system.

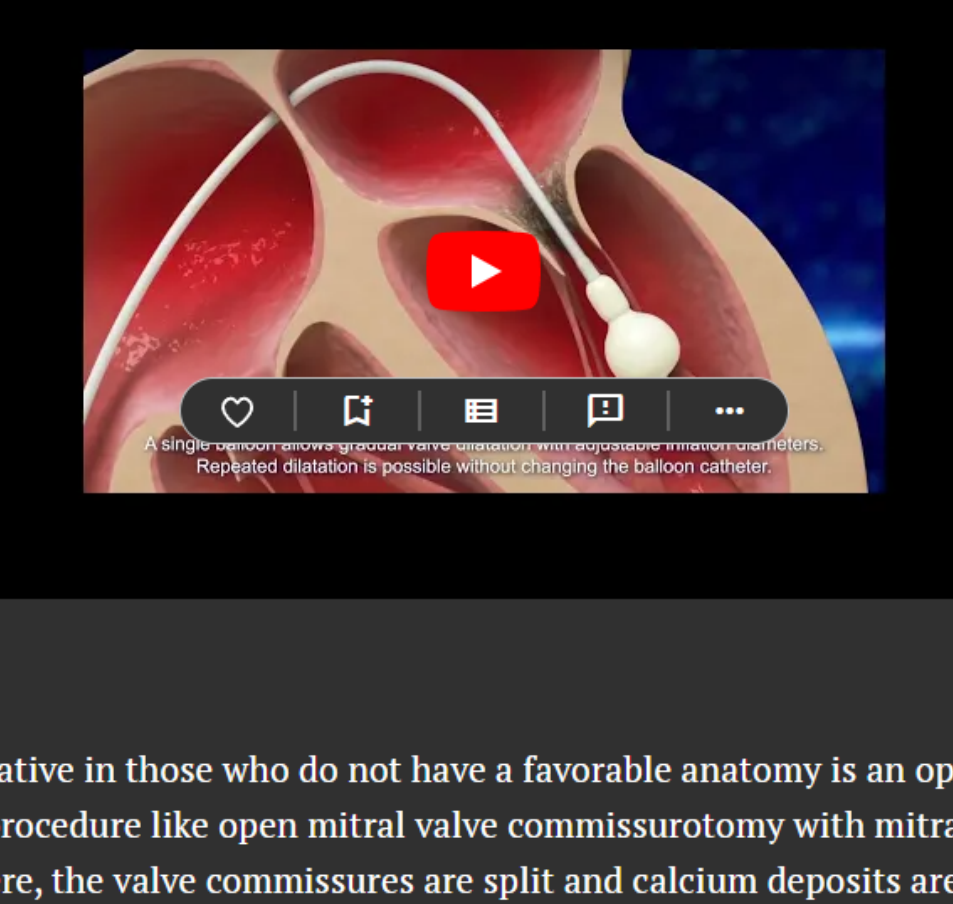
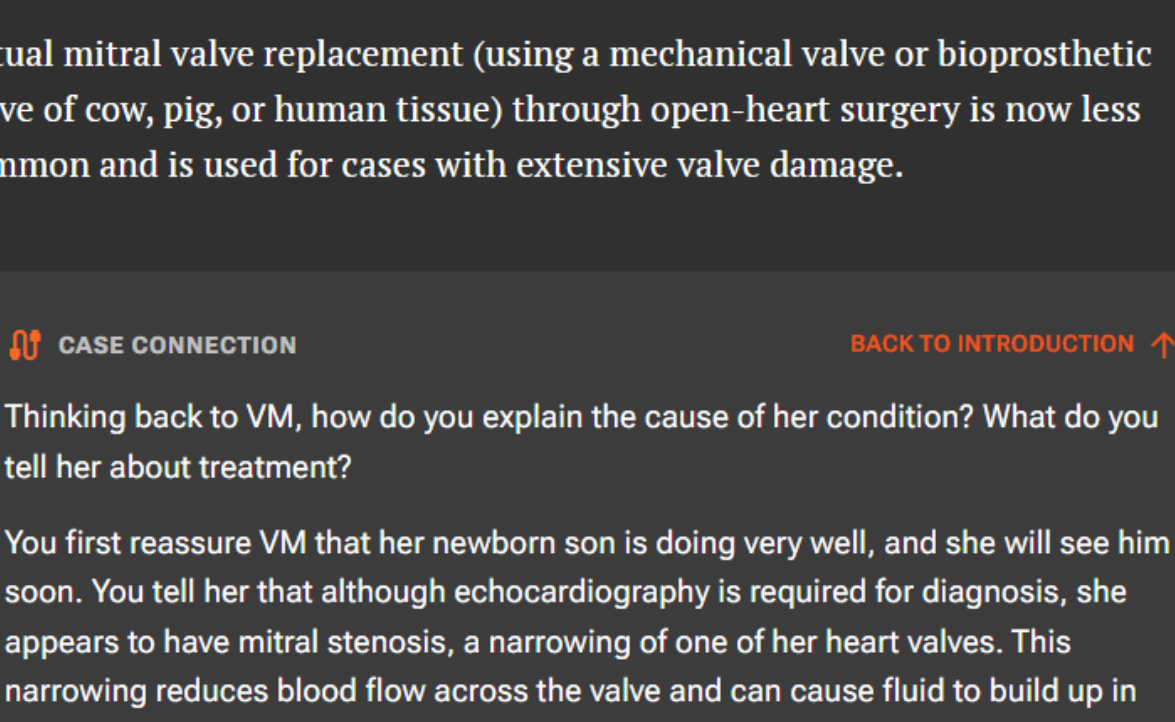


Figure 9. Percutaneous mitral balloon commissurotomy


Repeated dilation is possible without changing the balloon catheter

An alternative in those who do not have a favorable anatomy is an open surgical procedure like open mitral valve commissurotomy with mitral valve repair. Here, the valve commissures are split and calcium deposits are removed. this requires a much longer recovery than the percutaneous approach, as does valve replacement.

Actual mitral valve replacement (using a mechanical valve or bioprosthetic valve of cow, pig, or human tissue) through open-heart surgery is now less common and is used for cases with extensive valve damage.

> CASE CONNECTION BACK TO INTRODUCTION
>
> Thinking back to VM, how do you explain the cause of her condition? What do you tell her about treatment?
>
> You first reassure VM that her newborn son is doing very well, and she will see him soon. You tell her that although echocardiography is required for diagnosis, she appears to have mitral stenosis, a narrowing of one of her heart valves. This narrowing reduces blood flow across the valve and can cause fluid to build up in the lungs. You explain that MS is most often caused by rheumatic fever, an illness she likely had as a child. You reason that she developed pulmonary edema because the stenotic valve decreased the filling of the left ventricle during diastole and this is poorly tolerated with the large increase in blood volume during pregnancy. Also, th which further increases during the exertion of labor, decreases time for ventricular filling. You

Review Questions

1. Which of the following is not a cause of mitral stenosis?

- **A.** ○ Congenital mitral stenosis
- **B.** ○ Mitral annular calcification
- **C.** ● Myocardial infarction
- **D.** ○ Rheumatic heart disease

Hide Explanation

The correct answer is myocardial infarction (C). MI will not lead to stenosis of the mitral valve but may lead to mitral regurgitation due to papillary muscle rupture. Congenital Mitral Stenosis (A) is a condition in which patients are born with mitral stenosis (MS). Mitral annular calcification (B) occurs when the annulus, the ring-like structure holding the leaflets in place, becomes calcified and causes narrowing of the mitral valve. Rheumatic heart disease (D) is the most common cause of Mitral Stenosis and causes narrowing of the valve by inducing inflammation and scarring of the leaflets.

2. Which of the following is the most common symptom of mitral stenosis?

- **A.** ○ Cough
- **C.** ○ Diuretics
- **D.** ○ Heart surgery
- **E.** ● Percutaneous mitral balloon commissurotomy

Hide Explanation

The correct answer is Percutaneous Mitral Balloon Commissurotomy (E), which has a much faster recovery than surgical repair or replacement of the valve (D), which is reserved for more severe cases in which balloon commissurotomy is not an option. β-Blockers (A) and calcium channel blockers (B) can help control the heart rate, which may be elevated because atrial fibrillation or decreased forward flow, but they will not correct the valve itself. Diuretics (C) are medications that can help patients eliminate excess fluid, which can provide relief in patients with pulmonary edema. However, diuretics will not correct the problem with the mitral valve itself.

4. Which of the following describes the characteristic murmur of mitral stenosis?

- **A.** ● Diastolic murmur with an opening snap, followed by a low-pitched diastolic rumble at the apex
- **B.** ○ Harsh systolic crescendo-decrescendo murmur at the right sternal border, second intercostal space that radiates to the carotids
- **C.** ○ High-pitched, blowing, holosystolic murmur at the left fifth intercostal space at the midclavicular line radiating to the axilla
- **D.** ○ Soft, high ur at the left sternal border, third intercostal space,

diagnosis of all valvular disorders and assessing their severity. An electrocardiogram (A) is useful for evaluating patients with mitral stenosis for atrial fibrillation, as it measures the electrical activity of the heart. Magnetic resonance imaging (C) can be useful for evaluating the structure and anatomy of the heart but is a second-line diagnostic tool; echocardiography is still preferable. X-ray (D) provides a two-dimensional image of the heart and is less informative than echocardiography, but it may identify calcification of the mitral or aortic valves.

How would you rate this Brick?

References

- Dima C. Mitral stenosis. Medscape. Updated December 8, 2021. Accessed September 27, 2022. https://emedicine.medscape.com/article/155724-overview#a6
- *StatPearls*. StatPearls Publishing. Updated July 8, 2022. Accessed September 27, 2022. https://www.ncbi.nlm.nih.gov/books/NBK430742/